

Infection Control and Prevention

Hand Hygiene, PPE, and Sterilization

Regulation: 42 CFR §482.42

POLICY OVERVIEW

Infection Control and Prevention policies establish the framework for protecting patients, staff, and visitors from healthcare-associated infections (HAIs). These policies are among the most frequently cited during CMS surveys and are foundational to patient safety.

KEY REQUIREMENTS

1 Hand Hygiene Protocol

All clinical staff must perform hand hygiene using WHO-approved technique with alcohol-based hand rub or soap and water before and after patient contact, after contact with potentially infectious material, and before donning/after removing gloves.

2 Personal Protective Equipment (PPE)

Staff must select and use appropriate PPE based on transmission-based precautions. This includes gloves, gowns, masks/respirators, and eye protection. PPE must be properly donned, doffed, and disposed of per established protocol.

3 Sterilization & Disinfection

All reusable medical equipment must be properly decontaminated, cleaned, and sterilized or high-level disinfected according to manufacturer instructions and facility protocols before use on another patient.

4 Surveillance & Reporting

Ongoing surveillance of HAIs must be conducted. Reportable conditions must be submitted to appropriate public health authorities within required timeframes.

5 Isolation Procedures

Contact, droplet, and airborne precautions must be implemented appropriately. Signage, dedicated equipment, and private rooms (when available) must be utilized.

COMPLIANCE NOTES

The Infection Preventionist (IP) must have specialized training and devote sufficient time to the infection control program. A multidisciplinary Infection Control Committee should meet regularly and document outcomes.

COMMON SURVEY CITATIONS

Most commonly cited under §482.42(a) – Infection Control Officer qualifications, and §482.42(b) – Infection control policies and procedures.